

Common Denial Reasons & How to Respond

Why claims get denied · How to prevent them · SNF Therapy · 2026

Denials go directly to the ADR team — not to the DOR. Your role is **prevention**, not response. Most denials are caused by documentation issues that are entirely within your control. This guide explains why each denial happens and what you can do to make sure it never gets there.

The Most Common Denial Reasons

No Skilled Need Documented

Why it happens:

1 Notes describe WHAT was done but not WHY a licensed therapist was required. "Patient ambulated 100 ft" or "patient tolerated treatment well" does not establish skilled need.

How to prevent it:

Every note must answer: why did this patient need a licensed therapist today? Use the Skilled Need Language Guide. Document the clinical judgment, not just the activity.

No Physician Order on File

Why it happens:

2 Treatment began before a signed physician order was obtained, or the order expired and was not renewed before continued treatment.

How to prevent it:

Verify physician orders exist for every active Part B patient before treatment begins. Never treat without a signed order. Check orders on every new admission and payer transition.

Lack of Medical Necessity

Why it happens:

3 The diagnosis and functional deficits documented do not clearly support the level or type of treatment billed. Auditors cannot connect the treatment to the patient's condition.

How to prevent it:

Every note must link treatment directly to the patient's diagnosis and functional goals. Generic documentation that could apply to any patient is a red flag — individualize every note.

Frequency Not Justified

Why it happens:

4 Patient is seen 5x/week but documentation does not support why daily treatment was medically necessary vs a lower frequency.

How to prevent it:

Document individualized clinical rationale for every frequency level. Use the Frequency Justification Guide. All patients at the same frequency is a major audit trigger.

Plateau — No Progress Documented

Why it happens:

5 Auditor applies the improvement standard and determines the patient was not progressing. Note: post-Jimmo (2013), CMS cannot deny solely because a patient is not improving.

How to prevent it:

Document skilled need for maintenance explicitly per Jimmo standard. "No change" without maintenance justification is a direct denial trigger. State what decline would occur without skilled care.

Observation Stay — Wrong Payer Billed

Why it happens:

6 Patient was admitted as observation (not inpatient) — no qualifying hospital stay. Billed under Part A when it should have been Part B.

How to prevent it:

Verify admission status on every patient at admission. Observation status does NOT qualify for Part A SNF coverage. Always check — never assume inpatient status.

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Upcoded Evaluation Complexity		
	Why it happens:	How to prevent it:
7	A high complexity eval (97163/97167) was billed but documentation does not support the required complexity factors: 3+ comorbidities, complex history, high clinical decision-making.	Only bill high complexity when documentation clearly supports every complexity factor. Document each factor explicitly — do not assume the auditor will infer it from the diagnosis alone.
Duplicate Billing		
	Why it happens:	How to prevent it:
8	The same service was billed twice for the same patient on the same date, or the same units were submitted under two different codes incorrectly.	Review charges before submission. Ensure documentation matches exactly what was billed — code, units, and date of service. Flag discrepancies to billing immediately.
Reminder: Denials and ADR (Additional Documentation Requests) go directly to the ADR team — not to you. Your job is to make sure they never happen. Every denial on this list is preventable with the right documentation habits. If you receive any audit-related communication at the building level, contact your regional director immediately.		